

HC PAYER

US Healthcare and Insurance

- Healthcare is the organized system of providing medical services to maintain, improve, and restore people's health. It includes prevention, diagnosis, treatment.
- Health Insurance is a financial protection system that helps individuals pay for medical expenses.
- US healthcare system is a network of organizations, professionals, and government programs that provide medical care to the people.

Types of Health Insurance in the USA

1. **Employer-Sponsored Insurance** : Offered by employers to employees and their dependents. Premiums are shared between the employer and employee. This is the most common form of health insurance in the U.S.
2. **Individual/Family Plans**: Purchased directly by individuals from insurance companies. Can also be purchased through the Affordable Care Act (ACA) Marketplace. Lower-income individuals may qualify for government subsidies to reduce premium costs. Covers around 6% of the population.
3. **Public/Government Insurance**: Government-funded health insurance programs designed for eligible populations. Major programs include: Medicare , Medicaid , CHIP , Veterans programs ,Military health programs . These programs collectively cover around 37% of Americans.

Medicare: Medicare is a federal health insurance program primarily for people aged 65 and older, certain younger individuals with disabilities, and people with End-Stage Renal Disease

Medicare Parts

- **Part A** – Hospital insurance (inpatient care).
- **Part B** – Medical insurance (doctor visits and outpatient care).
- **Part C (Medicare Advantage)** – Private plans that combine Parts A and B and often include extra benefits such as dental ,vision .
- **Part D** – Prescription drug coverage.

Medicaid: Medicaid is a joint Federal and State government health insurance program for Low-income individuals and families, Pregnant women, Children, People with disabilities, Seniors requiring long-term nursing home care.

Services Covered

- Hospital services
- Doctor visits
- Laboratory tests
- X-rays
- Family planning services
- Long-term care

Medicaid Expansion is a provision of the Affordable Care Act that allows states to extend Medicaid coverage to more low-income adults. Income up to 138% of the Federal Poverty Level.

Dual Programs : Dual Programs are health insurance programs designed for individuals who qualify for both Medicare and Medicaid. These individuals are called Dual Eligible Beneficiaries.

Examples include:

- D-SNP (Dual Special Needs Plans)
- FIDE (Fully Integrated Dual Eligible Plans)

Children's Health Insurance Program (CHIP) : The program is a joint federal and state program which primarily covers child under 19 ensures children receive preventive healthcare and necessary medical treatment. The eligibility may vary based by state.

E.g. Routine health checkups, Doctor visits, Prescription medicines, X ray etc.,

Covers nearly 9 million children every year.

4. **Uninsured Population:** Around 8–9% of Americans do not have health insurance. They generally pay medical expenses out of pocket unless they qualify for assistance.

Affordable Care Act (ACA) : It is a U.S. federal law passed in 2010 to Expand health insurance coverage , Improve healthcare quality , Reduce healthcare costs also called Obamacare.

Marketplace Plans provide comprehensive health insurance coverage and help eligible individuals reduce healthcare costs through financial assistance.

<i>Premium Subsidies (Premium Tax Credits)</i>	<i>Cost-Sharing Reductions (CSR)</i>
• Reduce the monthly insurance premium. • Generally available to eligible individuals based on income. • Designed to make insurance more affordable.(100%-400%)	• Reduce out-of-pocket expenses such as: - o Deductibles - o Copayments - o Coinsurance • Available to qualifying lower-income individuals enrolled in eligible Marketplace plans.

Factors influencing :

- **Coverage Type** – Determines the kind of health insurance a member needs and helps match them with the appropriate plan (Medicare, Medicaid, Marketplace, etc.).
- **Age** – Influences eligibility for specific health plans, as certain programs are designed for particular age groups.
- **Income** – Affects eligibility for government programs, subsidies, and the overall cost of health insurance coverage.
- **Employment Status** – Determines whether a member can receive health insurance through an employer or needs individual coverage.
- **Life Events** – Major changes such as marriage, childbirth, divorce, job loss, or relocation may allow members to enroll in or change health plans outside the regular enrollment period.

Stakeholders:

Patients/Members – Individuals who receive healthcare services and are covered under health insurance plans.

Healthcare Providers – Doctors, hospitals, clinics, pharmacies, laboratories, and other organizations that deliver healthcare services.

Insurance Payers – Health insurance companies such as Molina, UnitedHealthcare, Aetna, Cigna, and Humana that finance and manage healthcare coverage.

Government Agencies – Federal and state organizations such as CMS, Medicare, Medicaid, CHIP, and public health departments that regulate and fund healthcare programs.

Policy Makers – Develop healthcare policies, laws, and regulations that govern the healthcare system such as eligibility rules, funding, compliance requirements etc.

Industry Suppliers - Provide products, services, and technology that support healthcare delivery and insurance operations. Supply medical equipment, pharmaceuticals, healthcare software, claims systems, and analytics solutions.

Molina Healthcare End to End payer Process

Molina is managed care organization in US. Its headquarters is in California, USA, founded in 1980 by Dr. C. David Molina.

The End-to-End Payer Process describes the complete workflow followed by a health insurance company (payer) to provide healthcare services to its members.

1. Member Enrollment (834)

The process starts with member enrollment. Individuals enroll in a health insurance plan, and their personal, eligibility, and coverage details are recorded in the payer's system. The enrollment information is typically exchanged electronically using the HIPAA 834 EDI transaction.

2. Provider Network Management

The payer builds a network of healthcare providers such as hospitals, physicians, clinics, laboratories, and pharmacies. Providers sign contracts with the insurance company to offer services to members at negotiated rates.

3. Member Services

Member Services assists members by answering questions related to benefits, eligibility, ID cards, claims status, provider information, and coverage details. This department serves as the primary point of contact for members.

4. Prior Authorization Process

Certain medical procedures, treatments, medications, or diagnostic tests require approval from the insurance company before they are performed. This process verifies that the requested service is medically necessary.

5. Healthcare service Delivery

After receiving approval, the healthcare provider delivers medical services to the member. These services may include doctor consultations, hospital admissions, surgeries, laboratory tests, or prescription medications.

6. Claim processing (837)

Once treatment is completed, the healthcare provider submits a medical claim to the payer requesting reimbursement. Claims are electronically transmitted using the HIPAA 837 EDI transaction. The payer validates the claim, checks eligibility, verifies benefits, applies policy rules, and determines the payable amount.

7. Payment and Remittance (835)

After claim adjudication, the payer issues payment to the provider and sends an Electronic Remittance Advice (ERA) using the HIPAA 835 EDI transaction. The remittance explains the payment details, adjustments, deductibles, copayments, or denial reasons.

8. Compliance and State Reporting

Health insurance companies must comply with federal and state healthcare regulations such as HIPAA guidelines. They regularly submit reports to regulatory authorities to ensure transparency and legal compliance.

9. Grievances and Appeals

If a member or provider disagrees with a claim decision, denial, or coverage determination, they can file a grievance or appeal. The payer reviews the request and makes a final decision after evaluating the supporting information.

10. Financial Management

The final stage involves managing the financial activities of the insurance company, including premium collection, provider payments, budgeting, auditing, and financial reporting. This ensures the organization's financial stability.

QNXT Functional Architecture: QNXT is a healthcare payer core administration system used by insurance companies (payers) to manage members, providers, claims, benefits, enrollment, billing, and payments from a single platform. It is widely used by health plans offering Commercial, Medicare, Medicaid, and Dual Eligible products.

- Processes and adjudicates medical claims.
- Manages appeals workflows.
- **Claims Intake (MCG):** Receives claims from providers, Processes paper and electronic claims, Validates claim data.

Medicaid enrollment process

State Agency (Zone 1):

- A State Agency (Medicaid agency) manages member enrollment into Medicaid health plans.
- It determines eligibility, allows plan selection, and sends enrollment data to payers using EDI 834.

Workflow

1.Award Contract : State agency awards contracts to health insurance payers. Authorize payers to provide Medicaid services.

2. Review Application : Reviews the member's Medicaid application. Verify applicant information.

3. Determine Eligibility : Checks if the member qualifies for Medicaid. Approve or deny eligibility.

4. Plan Selection : Eligible members choose a health plan. Assign preferred insurance plan.

5. Auto Assign Plan : If no plan is selected, the state automatically assigns one. Ensure the member receives healthcare coverage.

6. Send EDI File : Enrollment details are sent to the payer using EDI 834 (usually within 30–90 days). Officially enroll the member in the selected health plan.

Zone 2 – Payer Enrollment Processing

- After receiving the EDI 834 file, the payer (health plan) processes member enrollment.
- It creates member records, assigns a Primary Care Provider (PCP), and shares enrollment details with other healthcare vendors.

Workflow

1. Receives and validates the EDI 834 enrollment file. Verify enrollment data.
2. Creates a unique Member ID and enrollment record. Identify the member in the payer system.
3. Assigns a Primary Care Provider (PCP) to the member. Ensure the member has a primary doctor.
4. Welcomes the member and provides plan information. Help members understand and use their benefits.
5. Shares enrollment data with vendors like PBM, Dental, Vision, LTSS, Behavioral Health, Care Management. Enable members to receive all covered healthcare services.

Zone 3 - Reconciliation

- Reconciliation is the process of comparing State Agency and Payer enrollment records to ensure they match.
- It helps identify and correct enrollment differences.

Daily 834 Updates (834 UPD) :State sends daily enrollment changes through **EDI 834**. Update member demographics, eligibility, and coverage.

Monthly Reconciliation (RECON): State and payer compare enrollment records. Find and correct mismatched or missing data.

Annual Redetermination (ANN RD): Member's Medicaid eligibility is reviewed every year. Eligibility is renewed, modified, or terminated.

Plan Changes (PLN CHG): Members can change their health plan during the open enrollment period. Switch to a different health plan if eligible.

Disenrollment (DIS ENR): Member is removed from the health plan. Loss of eligibility, voluntary cancellation, relocation, or death.

Capitation (PMPM): PMPM = **Per Member Per Month** payment. State pays the health plan a fixed amount every month for each enrolled member. It Covers the cost of providing healthcare services.

Claims and Encounters

Claim processing:

- Claims are the detailed bill submitted by a doctor, Hospital to an insurance company asking for payment for services provided to a patient .
- In Molina we use MCG which intake application from clearing house vendor and our webportal application.
- It performs validation to ensure claims are billed correctly, it matches the claim to a QNXT member and provider, and then routes the claim to the appropriate QNXT environment.

Member → Provider → Diagnosis → Billing (837) → Clearinghouse → Payer → Adjudication → Payment (835) → Remittance/EOB → Review (if denied).

Step 1: Inbound Claim Processing

- Providers submit claims either as paper claims or electronic (EDI 837) claims.
- Paper claims are scanned using OCR and converted into electronic format.
- The File Transfer System (FTS) validates the files, checks for duplicate claims, and verifies the sender.
- The claim contains membe ID, Diagnosis code, Procedure Code, Date of service, Charges.

- A 997 Acknowledgement is sent to confirm that the claim file has been received successfully.

Step 2: Member & Provider Match

- The system matches the claim with the correct member and provider using details such as Member ID, Name, and Date of Birth.
- If a valid match is found, the claim continues for processing.
- If no match is found, the claim is sent to the Claims Editor.

Step 3: Claims Routing

- After matching, the claim is routed to the appropriate environment:
 - o **Medicare** – Active Medicare member.
 - o **Medicaid** – Active Medicaid member.
 - o **Claims Editor** – No member/provider match or inactive enrollment.

Step 4: Claims Editor

- Claims that fail automatic validation are manually reviewed.
- The examiner corrects missing or incorrect information.
- Corrected claims are loaded into QNXT for further processing.
- If the claim cannot be corrected, a denial notice is sent to the provider.

Step 5: Claims Processing

- The claim undergoes initial validation to check eligibility, required fields, and coding.
- It then enters automated adjudication, where business rules determine whether it should be approved, denied, or sent for manual review.
- Simple claims are automatically approved (Auto-Pay).
- Complex claims are pended for manual review by a claims examiner.

Step 6: Final Decision

- **Approved Claim:** Payment is calculated and sent to the provider along with an 835 Electronic Remittance Advice (ERA).
- **Denied Claim:** A denial notice explaining the reason is sent to the provider.
- After payment or denial, the claim processing is completed.

Encounters

Encounters are prepaid (capitated) claims submitted by providers to Molina. They are used for recording services provided, even though the provider has already been paid. All encounter files must follow HIPAA 5010 standards.

Supported Files

- 837 Institutional (I)
- 837 Professional (P)
- 837 Dental (D)

Response Files

- **999** – HIPAA compliance acknowledgement.
- **277CA** – Claim/Encounter acceptance or rejection status.

Inbound Encounter Processing

Inbound Encounter Processing receives encounter files from providers and validates them before loading them into the system. Ensure only valid encounter records are stored and processed.

process Flow

1. **Receive Encounter File (837 I/P/D)** from providers/vendors.
2. **HIPAA Validation** – Check HIPAA 5010 compliance.
 - a. **Fail** → Sent to Non-HIPAA process.
 - b. **Pass** → Loaded into the staging database.
3. **Member Match** – Verify the member exists.
4. **Provider Match** – Verify the provider exists.
5. **Validation Rule Engine** – Applies business rules, duplicate check, and void/replace validation.
6. **Editor Data Fix** – Claims with errors are corrected manually.
7. **ODS (Operational Data Store)** – Valid encounters are stored.
8. **277CA Notification** – Providers receive acceptance or rejection status.

Outbound Encounter Processing

Outbound Encounter Processing prepares validated encounter records and submits them to the state or federal agency.

process Flow

1. Extract finalized encounter records from ODS.
2. Perform state-specific validations.
3. Generate HIPAA 5010 837 encounter files.
4. Perform internal validation.
5. Submit files to the State/Federal Agency.
6. Receive response files (999, 277CA, or proprietary responses).
7. Automatically reconcile accepted and rejected encounters.
8. Correct rejected encounters (Void/Replace) and resubmit if required.

EDI & HIPAA

- HIPAA (Health Insurance Portability and Accountability Act)(1996). It is a U.S. federal law that protects the privacy and security of patient health information.
- The main objective of HIPAA is to ensure that Protected Health Information (PHI) is kept confidential and shared only with authorized individuals.
- PHI (Protected Health Information) includes patient details such as name, date of birth, address, medical records, diagnosis, treatment, insurance information, and payment details.
- Covered Entities include healthcare providers, health insurance companies, and healthcare clearinghouses.
- Failure to comply with HIPAA can result in legal penalties, fines, and disciplinary action.

EDI

- EDI (Electronic Data Interchange). It is the electronic exchange of business documents between organizations in a standard format.
- Eliminates manual paper-based transactions.
- Enables fast, accurate, and secure data exchange.
- The primary EDI standard used in U.S. healthcare is ANSI ASC X12.
- Healthcare transactions such as claims, eligibility, enrollment, and payments are exchanged through EDI.

Other standards :

- **EDIFACT** – International standard.
- **NCPDP** – Pharmacy transactions.
- **HL7** – Clinical and healthcare information exchange.

The EDI envelope contains:

1. ISA (Interchange Control Header)

- First segment, starts the EDI file.
- Contains Sender ID, Receiver ID, Date, Time, Version, Control Number.
- Starts the interchange.

2. GS (Functional Group Header)

- Groups similar transactions (e.g., multiple 837 claims).
- Starts the functional group.

3. ST (Transaction Header)

- Starts an individual transaction.
- Contains Transaction Type (e.g., 837) and Control Number.

4. Payload

- Contains the actual business data.
- Example: Patient details, diagnosis, procedure, claim amount.

5. SE (Transaction Trailer)

- Ends the transaction.
- Verifies segment count and Transaction Control Number.

6. GE (Functional Group Trailer)

- Ends the functional group.
- Verifies number of transactions and Group Control Number.

7. IEA (Interchange Control Trailer)

- Last segment; ends the EDI file.
- Verifies Functional Groups and Interchange Control Number.

- Every segment begins with an identifier (ISA, GS, ST, BHT, NM1, etc.).
- Each segment is written on a new line.
- Segments end with a segment delimiter (~).
- Data elements are separated by an element delimiter (*).
- Each segment contains related business information.

EDI Transaction Types

- **270/271** – Checks member eligibility and benefits.
- **837** – Submits healthcare claims to the payer.
- **276/277** – Checks claim status (pending, paid, denied).
- **835** – Sends payment details and remittance advice.
- **278** – Requests prior authorization before treatment.
- **834** – Enrolls or updates member insurance information.
- **820** – Transfers insurance premium payments

SNIP Level Edits (Strategic National Implementation Process)

SNIP(Strategic National Implementation Process) is a validation framework used to check whether HIPAA EDI transactions are correct. It helps identify and categorize errors in EDI files before claims are processed.

Type 1- Syntax Validation

- Checks whether the EDI file follows the correct X12 format.
- Prevents invalid EDI file formats.

Type 2 – HIPAA Compliance

- Verifies that the transaction follows HIPAA implementation guidelines.

Type 3 - Balancing

- Confirms totals, counts, and control numbers match throughout the file.
- Ensures data consistency and completeness.

Type 4 – Situation Testing

- Validates conditional fields based on business rules.
- Example: If a patient has insurance, insurance details must be present.
- Checks business logic.

Type 5 – External Code Set Validation

- Validates standard medical codes such as ICD-10, CPT.
- Prevents invalid diagnosis or procedure codes.

Type 6 – Product/ Line of Business Validation

- Ensures the claim matches the correct insurance product or plan.
- Example: Medicare, Medicaid, Commercial.
- Routes claims to the correct health plan.

Type 7 – Trading Partner Validation

- Checks payer-specific or trading partner-specific requirements.
- Ensures the claim meets payer-specific rules.

Member Portal

- A self-service web application that allows members to access and manage their health plan information.
- Helps members view account details, health records, make requests, and manage services online.
- Reduces dependency on customer support by enabling members to perform tasks themselves. Similar to an online banking portal where users can view and manage their accounts.

Features of the Member portal are:

- View and manage Member Profile.
- Access and download ID Card.
- View and update My Doctor (PCP) information.
- Access Health Records and medical information.
- View Plan Resources and coverage details.
- Track Wellness Programs & Rewards.
- Use Secure Messaging and Live Chat.
- Manage Family Accounts.
- Access useful health-related links and resources.

Provider Portal:

Provider portal same as member portal is helpful in finding the details of the patients. Availability means multi payer provider portal which means a member may not have the multi insurance coverages from different payers but may have different plans like medicaid and medicare with one payer like Molina. So the provider instead of looking one platform for each payer they can look them in one platform. There will be common functionalities or payer specific functionalities.

Payer spaces: Any requests regarding the eligibility claim or claim submission, the backend web services and the data particular member will be coming to the molina landscape.

The portal will reveal the member eligibility services and benefits, if the member is not eligible for some service, then provider will charge that particular service to member.

- **communication and collaboration feature:** where the providers can send some messages or the payer can notify providers on any of the new changes in the system.
- **The care coordination portal:** can be used to view the members under the provider and check for the members who need extra care.
- **Eligibility benefits screen:** provides the benefits of the particular member and whether he has any inactive or active coverage. We will have provider information such as pcp and plan maximums and deductibles.
- **Authorization dashboard:** shows the particular authorizations and what is the status such as approved,denied or pending.
- **Claims and remittance :** can check how many claims are submitted for particular member, start or end date and remittance of the payment.
- **Appeals:** can be submitted by the provider or member if the claim is not properly adjudicated or the amount is not paid.